



THE REPUBLIC OF UGANDA
MINISTRY OF HEALTH

THE MALARIA VACCINE



FREQUENTLY ASKED QUESTIONS

1ST EDITION



THE MALARIA VACCINE

INTRODUCTION

Malaria continues to bear a big burden in Uganda both socially and economically. Malaria still remains the highest contributor to the all causes of under 5 mortality and is responsible for 14% of the deaths that occur among children less than 5 years in our country. Uganda is the third highest contributor to the global malaria cases at 5.1% (2022) and the 8th highest contributor to the global malaria deaths at 3.2% (according to the World Malaria Report 2022).

So, the introduction of the Malaria vaccine into the routine immunization program is one of the key milestones that Uganda has achieved in our efforts to have a Malaria free Uganda by 2030.

1. What causes malaria?

Malaria is caused by a parasite called Plasmodium, and it is transmitted to humans through the bite of the infected female Anopheles mosquito.

2. What are the signs and symptoms of malaria?

- The most common symptoms of malaria include fever, headache, chills, and vomiting
- In severe cases, malaria can lead to anemia, difficulty in breathing, organ failure or even death.

3. How is malaria transmitted?

Malaria is primarily transmitted through the bites of infected female Anopheles mosquitoes. The mosquitoes become infected by biting a person with malaria and can then spread the parasite to others.

4. Who is most at risk of Malaria?

Everyone is at risk of Malaria but children under five years are more at risk of the disease. Those below two years are more at risk of getting severe forms of the disease

5. What are the malaria prevention and control measures.

The malaria prevention and control measures include correct and consistent use of insecticide treated mosquito nets, indoor residual spraying, Intermittent Preventive Treatment during pregnancy, prompt diagnostic testing and treatment of confirmed cases with effective anti-malarial medicines and vaccines.

The government has introduced the Malaria vaccine into the routine immunization schedule to prevent severe Malaria and death

6. How does the malaria vaccine work?

The malaria vaccine works by stimulating the immune system to produce antibodies that prevent *Plasmodium falciparum* parasites from infecting liver cells, where they mature and multiply. This stops the infection from progressing and causing severe illness.

7. How effective is the malaria vaccine?

The malaria vaccine offers adequate protection against severe malaria and death. It has proven to significantly lower the number of severe malaria cases, reduce number of hospitalization admissions, and reduce deaths among fully vaccinated children (four doses).

The vaccine's impact increases when it is combined with other malaria preventive measures, e.g., the use of insecticide-treated bed nets (ITN), seasonal malaria chemoprevention (SMC), perennial malaria chemoprevention (PMC) and indoor residual spraying (IRS), and early health seeking behaviors among others.

8. How safe is the vaccine?

- The malaria vaccine is safe. It has undergone rigorous testing in clinical trials involving over 800,000 children across multiple African countries. Over 6,000,000 children have received the vaccine since it was introduced in the five countries of Malawi, Kenya, Cameroon, South Sudan and Ghana since 2019.

- The vaccine has been approved by the World Health Organization (WHO)

It has also been approved by the National Drug Authority as ready for use in Uganda.

9. Who is eligible to receive the malaria vaccine?

Children below one year of age starting at six months

10. Why do we start vaccinating children from 6 months?

Because that is when they need it most since they have lost the acquired immunity from the mother.

11. How many doses of the vaccine will the child receive and at what intervals?

A child will receive 4 doses of malaria vaccine, starting at 6 months, 7 months, 8 months, and at 18 months. Completion of the 4 doses gives the best protection against severe Malaria.

12. What happens if my child does not complete the required malaria vaccine doses?

The malaria vaccine reduces on the number of malaria episodes in children and severity of the disease. Therefore, if the child is not fully vaccinated, he/she stands high chances of suffering from severe malaria and may die of malaria when exposed. It is important for the child to complete all the 4 recommended doses in addition to all Malaria prevention and control measures.

13. After one has been vaccinated, can he/she still catch malaria or infect others?

Yes. Children who get vaccinated can still get malaria however the vaccine reduces their chances of getting severe forms of malaria. To prevent malaria, the children should continue using other malaria interventions like sleeping under treated mosquito nets. In case of any fever seek medical attention.

14. How is the vaccine administered?

The vaccine is by injection on the right thigh

15. Do you first test the children for malaria before administering the vaccine?

No

16. Can a sick child be vaccinated?

No. If a child is sick with fever, the child should be tested and treated first. When the child is better, he or she can be vaccinated.

17. Where can one access the malaria vaccine?

The malaria vaccine will be available in all health facilities (public and private) providing routine immunization services and community outreach posts

18. What are the side effects of the malaria vaccine?

- The malaria vaccine is generally well-tolerated, with the most common side effects being mild and temporary, such as soreness at the injection site, mild fever, or fatigue. These symptoms usually resolve within a day or two.
- Care givers are also advised to immediately seek medical assistance in case of any medical concern related to vaccination.

19. How should the vaccine be stored?

The vaccine is heat sensitive and should therefore be stored within temperatures below 20 -80 C like other vaccines.

20. Why is the malaria vaccine important?

The malaria vaccine is a critical tool in the fight against malaria, and one of the leading causes of illness and death among children under five in Uganda. By reducing the risk of severe malaria and hospitalization, the vaccine saves lives and alleviates the burden on healthcare systems.

21. How long does the protection from the vaccine last?

Protection from the malaria vaccine lasts for about 3-4 years after the final dose. However, studies are ongoing to determine whether additional booster doses will be needed to maintain immunity.

22. Can a child get malaria even after being vaccinated?

Yes, a vaccinated child can still get malaria, but the vaccine significantly reduces the risk of severe disease, hospitalization, and death. It is important to continue using other preventive measures, such as sleeping under a mosquito net every night, and seeking timely treatment among others.

23. When will the vaccine be made available in Uganda

The vaccine will be rolled out in April 2025.

24. Is the malaria vaccine free?

Yes, the malaria vaccine like other existing vaccines is free of charge.

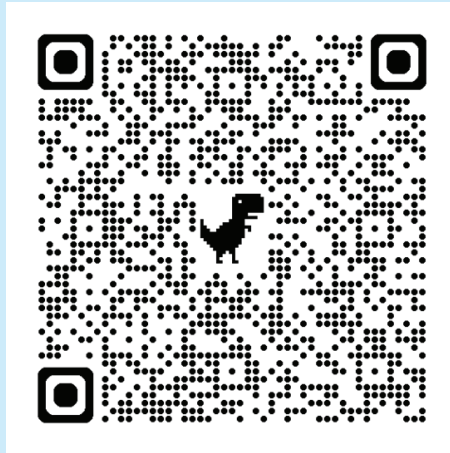
25. Does the malaria vaccine replace other malaria prevention methods?

No, the malaria vaccine is an additional tool in the fight against malaria and should be used alongside other preventive measures such as sleeping under insecticide-treated nets, using indoor residual spraying, seeking care within 24 hours of seeing signs; and taking antimalarial medications when needed.

26. Can a child get the malaria vaccine along with another vaccine on the same day?

Yes, a child can receive the malaria vaccine alongside other vaccines on the same day.

There is no harm receiving multiple vaccines according to the recommended schedule. Research has shown that administering new vaccines along with the existing vaccines is safe and operationally feasible.



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